# Assessment 2: Critical Review on Health Across the Lifespan

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## Critical Reflections on Health Across the Lifespan

### Module 3 Young Adulthood: Adolescent Mental Health and Suicide Risk

Calear et al. (2026) examined multilevel risk and protective factors for self-harm, suicidal ideation and suicide attempts among Australian adolescents. Using data from 4,122 students attending 134 schools, the prospective study found that mental health difficulties, peer problems, female or gender-diverse identity and LGBTQA+ identity were associated with self-harm or suicidal ideation. Unexpectedly, greater prosocial behaviour was also associated with self-harm and suicidal ideation, while peer problems were associated with suicide attempts. A strength of the study is its large Australian cohort and longitudinal design, which allowed risk factors to be examined over a 12-month period. It considered individual, family, social and school-level influences.

However, the findings should be interpreted cautiously. Only 4,122 of the 5,720 students who completed the baseline assessment participated at follow-up. Attrition was higher among students with poorer mental health and greater suicidality, which may have led to an underestimation of risk. In addition, the outcomes were primarily measured through adolescent self-report, and the observational design identified associations rather than proving causation. The unexpected prosocial behaviour association was not significant in univariate analyses, so it may reflect model complexity rather than a direct relationship. Therefore, the study can inform early identification priorities, but it cannot demonstrate that a particular screening or nursing intervention will reduce suicide attempts. The study also provides limited evidence about how nurses should respond after a young person is identified as being at risk.

This article connects with ULO 1 because it demonstrates that health and wellbeing are influenced by psychological, social and environmental factors. It relates to ULO 3 by highlighting the importance of respectful, inclusive assessment for gender-diverse and LGBTQA+ adolescents, while avoiding assumptions or stigma. ULO 4 is demonstrated through its focus on adolescence as a developmental period involving changing emotional, social and cognitive needs. For nursing practice, the findings reinforce the value of non-judgemental communication and holistic psychosocial assessment, including peer relationships, bullying, family support and protective factors. I would use structured assessment to support early recognition, but would not

rely on a checklist alone. Appropriate safety planning, referral and follow-up are also essential.

## Module 4 Middle Adulthood: Neighbourhood Context and Psychological Distress

Learnihan et al. (2026) examined whether neighbourhood social fragmentation was associated with psychological distress among adults aged 40–65 in Brisbane. Using four waves of the HABITAT longitudinal study (2009–2016), researchers linked survey responses from 2,902 men and 3,950 women with census-derived neighbourhood data. After adjustment for individual, household and neighbourhood factors, men living in the most fragmented areas had higher distress scores than those in the least fragmented areas. The difference was small (0.66 points on the 24-point Kessler-6 scale). For women, the association was not statistically significant after neighbourhood disadvantage was included. A strength is the Australian longitudinal design, which considered neighbourhood context alongside individual characteristics.

The results warrant caution. The small difference may have limited importance for individual clinical care, although it may matter at a population level. The neighbourhood index describes area characteristics; it does not directly measure participants' social relationships, loneliness or sense of belonging. Although repeated observations strengthen temporal analysis, this observational study cannot show that fragmentation caused distress. In addition, the sample included people who remained in their neighbourhood for at least seven years, so findings may not apply to more mobile residents. The Kessler-6 focuses on internalising symptoms and may not capture how distress is expressed by all men, a limitation acknowledged by the authors. The sex-specific finding should not be interpreted as evidence that fragmented neighbourhoods affect only men.

This article connects with ULO 1 by showing that health is shaped by social and environmental conditions as well as individual factors. It relates to ULO 3 through attention to community context and the need to avoid treating mental distress as solely an individual failing. ULO 4 is relevant because the study focuses on midlife, when work, family and caring demands may intersect with neighbourhood resources. For nursing practice, the findings support asking about social connection, community resources and environmental barriers when assessing wellbeing. However, the study does not test a nursing intervention, so it cannot

establish that community programs will reduce distress. Nurses can use this evidence to inform holistic assessment and advocacy while tailoring support to each person's needs. Reflecting on this, I realise that the social environment in which people live can shape wellbeing as strongly as individual coping strategies, and this strengthens my understanding of the importance of holistic, community-aware nursing assessment.

**Module 5 Older Adulthood: Social Media, Hearing Loss and Social Connection**

Jayakody et al. (2022) explored Australian older adults' views on creating a Facebook community to support the psychosocial wellbeing of hearing-impaired older people. In a community conversation, 40 adults aged over 60 discussed design and content for a proposed online group. Most participants did not use Facebook, were not interested in joining, and preferred telephone calls or in-person activities. They also raised concerns about digital confidence, internet access, privacy and usability. A strength is the community-led approach: gathering views before designing an intervention can prevent services from being built around assumptions about older adults' needs.

However, the findings should not be interpreted as evidence that social media does or does not reduce loneliness. The study explored views about a proposed Facebook group; it did not implement the group or measure changes in loneliness, social isolation or wellbeing. The 40 participants may not represent all Australian older adults or people with hearing loss, particularly those who already use digital platforms or are more socially isolated. The group discussions may also have encouraged shared responses, and the article provides limited evidence about whether participants' views differed by age, hearing needs or digital access. Consequently, the findings identify barriers and preferences, but cannot establish the effectiveness of alternative interventions.

This article connects with ULO 1 because it distinguishes objective social contact from people's own perceptions of connection and wellbeing. It relates to ULO 3 through respect for older adults' preferences, autonomy and dignity; participants specifically cautioned against condescending language. ULO 4 is relevant because hearing loss and ageing can shape communication needs and access to social participation. For nursing practice, the study cautions

against prescribing digital connection as a universal solution. I would ask each person what forms of contact are meaningful and accessible, and consider practical barriers such as hearing accommodations, internet access, privacy and digital literacy. Phone calls and face-to-face community activities may be more acceptable for some people. Digital options should be offered alongside, not instead of, approaches chosen with the older person.

## References

Calear, A. L., Batterham, P. J., Werner-Seidler, A., Maston, K., Torok, M., O'Dea, B., Larsen, M. E., & Christensen, H. (2026). Multilevel risk and protective factors for self-harm, suicidal ideation and suicide attempt in adolescents. *Journal of Child Psychology & Psychiatry, 67*(5), 609–619. https://doi.org/10.1111/jcpp.70024

Jayakody, D. M. P., Tan, Y. M. E., Livings, I., Costello, L., Flicker, L., & Almeida, O. P. (2022). Australian older adults' views on using social media for reducing social isolation and loneliness in hearing impaired older adults: A community conversation. *Australasian Journal on Ageing, 41*, 585–589. https://doi.org/10.1111/ajag.13137

Learnihan, V., Bagheri, N., & Turrell, G. (2026). The longitudinal association between living in socially fragmented neighbourhoods and psychological distress among middle aged adults in Brisbane, Australia. *Journal of Affective Disorders, 401*, Article 121318. https://doi.org/10.1016/j.jad.2026.121318

## GenAI Acknowledgement Statement

**Option 2:** I did not use Generative AI in the preparation of this assessment. I confirm that the content of this critical reflection is entirely my own work, and that no part of it was generated, rewritten, or substantially improved by generative artificial intelligence tools.